

Facultad de Ingeniería y Negocios
Universidad de Las Américas
Santiago, Chile
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Editorial Office
Data (MDPI)

Re: Data Descriptor submission - “Chile-ED-Resp: A Curated and Reproducible Weekly Hospital Dataset for Respiratory Emergency-Demand Forecasting in Chile”

Dear Editors,

We are pleased to submit the above Data Descriptor for consideration in *Data*. The manuscript describes a secondary research dataset and reproducible processing pipeline derived from the official Chilean Ministry of Health/DEIS resource *Atenciones de urgencias de causas respiratorias por semana epidemiológica*, produced through the Sistema de Atención Diaria de Urgencias (SADU). The release covers the closed 2022–2025 period and focuses on hospital emergency units. It preserves official respiratory-demand counts and age strata while adding only deterministic provenance, quality-control, coverage, and time-series features.

The contribution is aligned with the Data Descriptors section because its emphasis is reusable data infrastructure rather than the superiority of a forecasting model. The public package includes a curated long table, a hospital-week forecasting table, a machine-readable dictionary, source and release manifests, SHA-256 checksums, hospital-coverage and cause inventories, validation reports, unit tests, and a GitHub Actions workflow that rebuilds the release from the official Parquet resource.

Repository: <https://github.com/cvidalmsu/chile-ed-resp-benchmark>

The tagged Version 1.1.0 release is permanently archived in Zenodo at <https://doi.org/10.5281/zenodo.22209795>, and the version-specific DOI is included in the manuscript. The official portal links the source data to Creative Commons Attribution-NonCommercial 2.0 Generic (CC BY-NC 2.0); the curated data preserve that licensing condition, whereas the processing software is released under the MIT License.

All authors have approved the manuscript and agree with its submission to *Data*. The manuscript is not under consideration elsewhere. The dataset uses aggregate public administrative statistics and contains no patient-level or directly identifying personal information.

We believe Chile-ED-Resp can provide a transparent and reusable foundation for hospital-demand forecasting, health-informatics teaching, and reproducible secondary analysis of respiratory emergency activity in Chile. Thank you for considering our submission.

Sincerely,

Cristian Vidal-Silva, Ph.D.

on behalf of all authors

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